

health corporates. But how crucial a role are the PE firms playing in the healthcare boom in India? "PE firms invested Rs 4,500 crore in Indian healthcare between 2006 and 2010," says Krishnakumar, executive director, private equity syndicator, Avendus Capital. Sandeep Singha, MD of Sequoia Capital India, which has invested Rs 450 crore into healthcare, agrees: "PE is crucial. Healthcare requires capital and unless you have PE funding, the evolution of the sector will be slower." Also, healthcare is a high-growth area that does not get impacted by an economic downturn. "It's a huge market," points out Singha, "that will be growing for the next 50 years and the market opportunity is staggering." Rs 900 crore of PE has been invested into Indian healthcare companies in 2010.

It's the surge of private equity in healthcare that's bringing in young entrepreneurs, often from unexpected backgrounds. Take Dr Dharminder Nagar. He started work on Paras Hospitals, Gurgaon in 2005. The Rs 70-crore venture required for this dream project was managed through a debt equity ratio of 1:1. "My father, Ved Ram Choudhary, was a philanthropist and the founder chairman of Paras Group of Industries," he says. From a humble beginning as the owner of a dairy, Paras, he became the largest milk producer and exporter, with interests in real estate across north India. Nagar worked as a doctor in the UK for a while and later joined the Imperial College, London, to study hospital

and health systems. "It's this first hand experience as a practitioner in London hospitals that inspired me to bring international quality healthcare to India," he adds. Paras Hospitals started in Gurgaon in 2005. Within five years, the group has grown 30-40 per cent year on year and acquired a multispecialty hospital in South Delhi.

With easy access to visa facilities, medical tourism is turning out to be the other potential source of income for the healthcare industry. "It's a growing segment in India, with over 2 to 2.5 lakh annual travellers to India increasing at a rate of 30-35 per cent," says Charu Sehgal, head of life sciences and healthcare at Deloitte Touche Tohmatsu India. Most large healthcare providers are increasingly focusing on attracting medical tourists. "Apart from generating higher revenues for the hospital—to an Indian patient—a medical tourist also spends 2-3 times more than a normal tourist in the country." "It is definitely a segment that the big players have in mind as they make their expansion plans," says Sehgal.

New Ethics, New Quality

After all the fuss he made about building his hospital, Dr Ramakanta Panda is left with a smile on his face today. In a resounding testimony to his commitment to quality health delivery, AHI is winning praises and kudos. "Healthcare business was a bit of a Munnabhai earlier," he jokes. "There

Doing Good Profitably

Dr Ashwin Naik

Dr Ashwin Naik, 37, and Dr Veerendra Hiremath, 35, set up Vaatsalya, a unique model of affordable hospital network in under-served tier II and III towns. They decided to address this demand-supply gap. Vaatsalya runs at eight locations in Karnataka, focusing on the mother and child, offers about 520 beds and sees 20,000 patients a month.



"Doctors from rural districts rarely go back to their roots."

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CARDIOLOGY IS SHAPING UP AS A PRIME AREA OF HOSPITAL EXPANSION

was no format or organised approach. Today, even hygiene value has gone up exponentially." Experts agree. To Dr Sachin Wagh, with his experience in hospital management, the main reason behind new-look hospitals is that planning and designing are now widely available to promoters. "A simple brief we recently received was that the hospital should not

look like a hospital." The spectrum of catering, food and dietetics have gone up, along with the processing of linen as an integral component of infection control. "Almost all newer hospitals opt to outsource support services—dietary, laundry, housekeeping and security," he adds. Apart from medical technology, hospitals are investing in elec-